MSD MANUAL
Consumer Version

Urinary Incontinence in Adults

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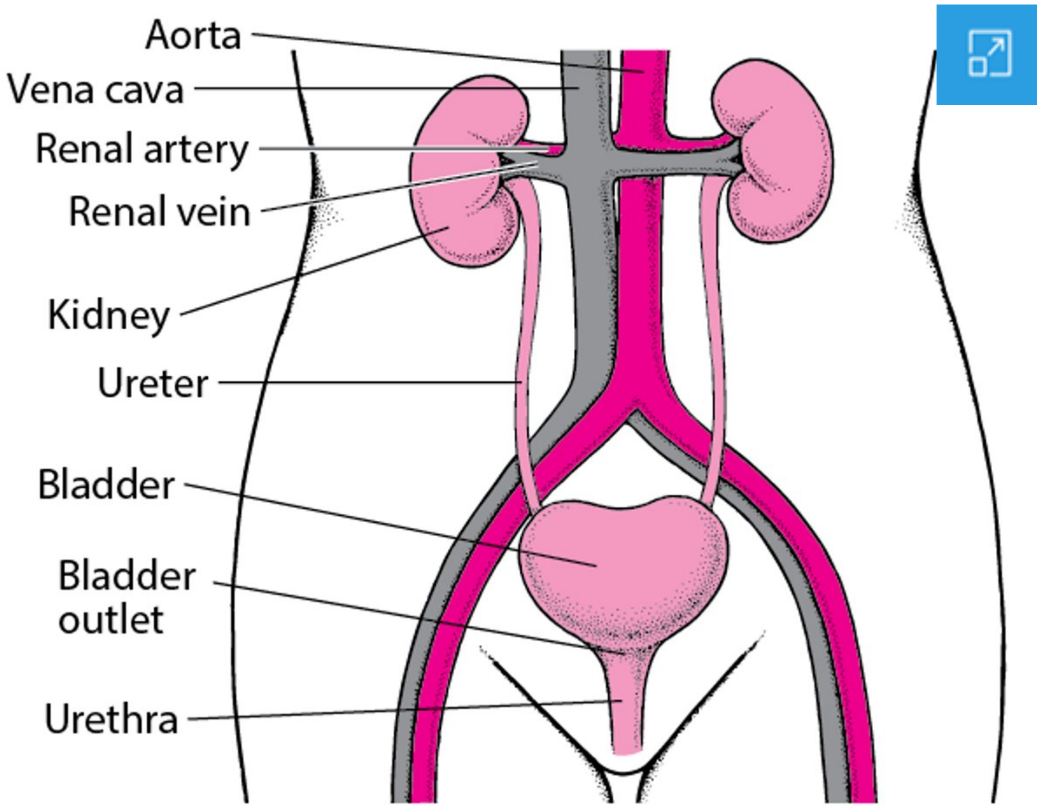
Urinary incontinence is involuntary loss of urine.

The Urinary Tract



Incontinence can occur in both men and women at any age, but it is more common among women and older adults, affecting about 37% of older women worldwide and 15% of older men in the United States. Although incontinence is more common among older adults, it is not a normal part of aging. (See also Control of Urination.) Incontinence may be sudden and temporary, as when a person is taking a medication that has a diuretic effect, or it may be long lasting (chronic). Even chronic incontinence may sometimes be successfully treated.

Urinary incontinence in children is discussed separately.

Types of Incontinence

There are several types of incontinence:

- **Urge incontinence** is uncontrolled urine leakage (of moderate to large volume) that occurs immediately after an urgent, irrepressible need to urinate. Getting up to urinate during the night (nocturia) and nocturnal incontinence are common.
- **Stress incontinence** is urine leakage due to abrupt increases in intra-abdominal pressure (for example, those that occur with coughing, sneezing, laughing, bending, or lifting). Leakage volume is usually low to moderate.
- **Overflow incontinence** is dribbling of urine from an overly full bladder. Volume is usually small, but leaks may be constant, resulting in large total losses.
- **Functional incontinence** is loss of urine because of a problem with thinking or a physical impairment unrelated to the control of urination. For example, a person with dementia due to Alzheimer disease may not recognize the need to urinate or not know where the toilet is. People who are bedridden may be unable to walk to the toilet or reach a bedpan.

Often, however, a person has more than one type of incontinence. People are then described as having mixed incontinence.

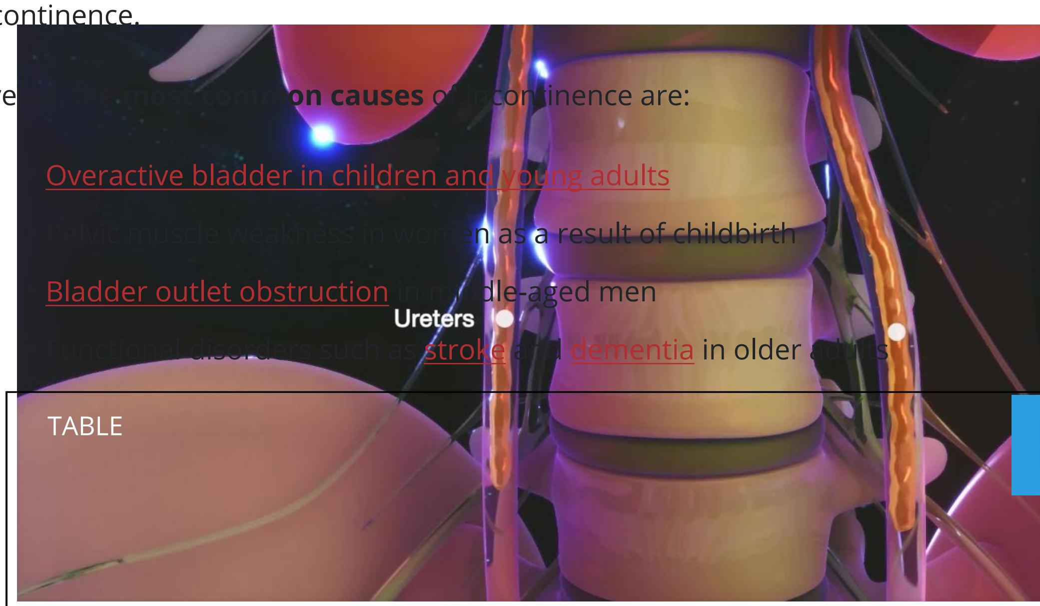
Causes of Urinary Incontinence

Several mechanisms can lead to urinary incontinence. Often, more than one mechanism is present:

- Weakness of the urinary sphincter or pelvic muscles (called bladder outlet incompetence)

Weakness or underactivity of the bladder wall muscles, bladder outlet obstruction, or particularly both can lead to inability to urinate (urinary retention). Urinary retention can paradoxically lead to overflow incontinence because of leaking from an overly full bladder.

An increase in the volume of urine (for example, caused by diabetes, use of diuretics, or excessive intake of alcohol or caffeinated drinks) can increase the amount of urine lost with incontinence, trigger an episode of incontinence, or even cause temporary incontinence to develop. However, it should not cause chronic incontinence. Functional problems, which are conditions that affect the function of other organs in the body, commonly increase the volume of urine lost among people who are incontinent. However, functional problems are rarely the only cause of permanent incontinence.



Overall, the **most common causes** of incontinence are:

- Overactive bladder in children and young adults
- Pelvic muscle weakness in women as a result of childbirth
- Bladder outlet obstruction in middle-aged men
- Functional disorders such as stroke and dementia in older adults

TABLE

- Something blocking the exit path of urine from the bladder (called bladder outlet obstruction)
- Spasm or overactivity of the bladder wall muscles (sometimes called overactive bladder)
- Weakness or underactivity of the bladder wall muscles
- Poor coordination of the bladder wall muscles with the urinary sphincter
- An increase in the volume of urine
- Functional problems

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Some Mechanisms of Incontinence

Mechanism	Examples
Weakness of the urinary sphincter or pelvic muscles (bladder outlet incompetence)	Atrophic urethritis, atrophic vaginitis, or both Medications Pelvic muscle weakness (for example, caused by having had several vaginal deliveries or pelvic surgery) Prostate surgery (most often complete removal of the prostate)
Blockage (bladder outlet obstruction)	Prostate enlargement (benign prostatic hyperplasia) or cancer Bladder stones Impacted stool Medications
Overactivity of bladder wall muscles (overactive bladder)	Bladder irritation (for example, caused by infection, stones, or rarely cancer) Disorders that can affect brain centers that control urination (such as stroke, dementia, or multiple sclerosis) Cervical spondylosis or spinal cord dysfunction (which can put pressure on the spinal cord and thus impair bladder function) Bladder outlet obstruction
Underactivity of bladder wall muscles	Nerve damage (for example, by herniated discs, other spinal cord disorders, surgery, tumors, injury, diabetes, or alcohol use disorder)